

# Getting Ready for Your Guided AI Session

*A short guide to read before you begin*

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This guide explains what will happen when you sit down for your session with the AI assistant. Reading it first means you'll know what to expect, why the session is shaped the way it is, and how to get the most out of it. *It takes just a few minutes to read.*

## What this session is

You are taking part in a research study on **Trial-Based Cognitive Therapy (TBCT)**, an approach that helps people understand the link between what happens to them, what they think, how they feel, and what they do.

In this session you'll work through one specific part of TBCT with an AI assistant that acts as a guide. Its job is to walk you through a simple, well-tested exercise so you can see how these pieces connect — first with a neutral example, and then, gently, with an example from your own life.

**Good to know:** There are no right or wrong answers here, and nothing you say is being graded. This is about noticing and exploring together — not about performing, judging yourself, or fixing anything on the spot.

## Who you'll be talking to

You'll be chatting with an AI assistant that has been set up to guide this one TBCT exercise. Think of it as a friendly demonstrator for this particular module.

### A few things to keep in mind:

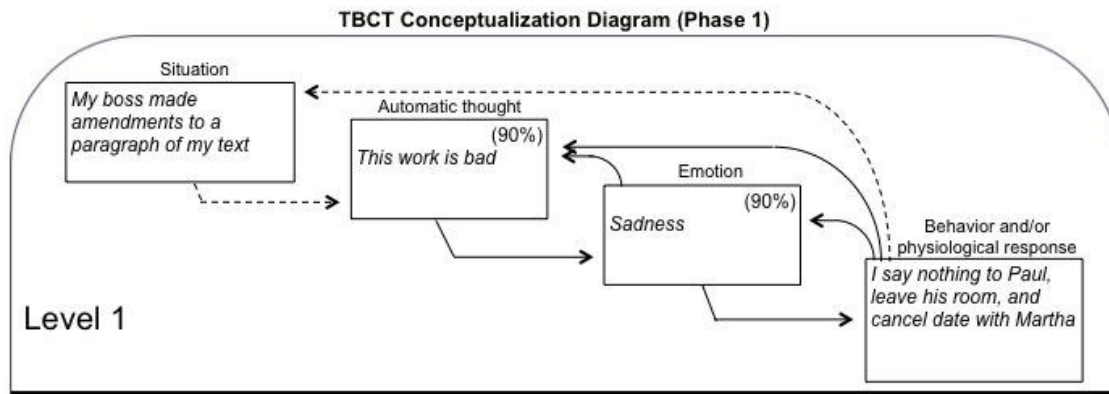
- The AI is not a replacement for your therapist or doctor. It's guiding one exercise, not managing your overall care.
- It won't give advice or tell you what to do. Instead it asks questions and lets you arrive at your own insights.
- It stays focused on this one topic. If you ask about other techniques or unrelated matters, it will gently keep to the exercise.

## How the session will flow

The session moves through a clear sequence. You don't need to memorise it — the AI leads the way — but here's the shape so nothing feels out of the blue.

### Part 1 — Seeing how the cycle works

You'll explore a simple idea: a situation triggers an automatic thought, that thought sparks an emotion, the emotion shapes how you behave and how your body reacts — and then those reactions loop back and feed the whole cycle again. The diagram below is the map you'll be working from:



*The TBCT Conceptualization Diagram, Level 1 — shown here with an example. Copyright: Irismar Reis de Oliveira; trial-basedcognitivetherapy.com.*

To make this clear, the AI will first walk you through a **neutral example about three job candidates** — three different people who hear exactly the same compliment in an interview, yet each one thinks, feels, and reacts completely differently.

**Why start with strangers, not you?** It's much easier to see the pattern clearly on a neutral example first. Once you've seen how it works with the three candidates, looking at your own situation becomes far simpler — and less overwhelming. So if you're wondering “why are we talking about interview candidates?”, that's exactly the point, and your own example comes right after.

After the three-person example, the AI will come back to **a situation from your own life** and help you look at it through the same lens.

## Part 2 — Meeting the “cognitive distortions”

Some automatic thoughts are accurate, but others are exaggerations or errors in thinking — these are called cognitive distortions. You'll use the list included at the end of this guide (the Annex), read a few of them, and together look at whether any seem to fit the thoughts you noticed in Part 1.

**The AI won't tell you which distortion you have.** It will ask whether any on the list seem to fit — the recognition is yours to make.

## Closing

At the end, the AI will invite you to keep noticing your automatic thoughts day to day using the distortions list, and will mention a technique your therapist may introduce in a later session. That deeper work isn't done here — this session is just the first step.

## What the conversation will feel like

- **Lots of questions.** The AI guides mostly by asking, not explaining. This is on purpose — the insights land better when they're yours. If it feels like it's “not just telling you the answer,” that's the method working.
- **Unhurried.** You can take your time. There's no clock pressuring you and no need to have polished answers ready.
- **Okay to be unsure.** If something feels abstract or confusing, just say so. The AI will slow down or try a different angle.
- **Conversational.** It's meant to feel like a real conversation, not a lecture or a quiz.

## What to have ready before you start

1. **The cognitive distortions list.** You'll need it for Part 2. It's included at the end of this guide (see the Annex) — have it open or printed nearby. The AI will check that you have it before that part begins.
2. **A real but manageable example.** It helps to have in mind a situation from your life that bothered you — something real, but not the most painful thing you can think of. A moderate, everyday example works best for learning the method.
3. **A quiet moment.** Set aside a stretch of uninterrupted time where you can focus and reply thoughtfully.
4. **An open, curious frame of mind.** You're here to observe how thoughts, feelings, and behaviours connect — not to judge or change them right away.

## A few tips to get the most from it

- Answer honestly — the exercise only reflects back what you put into it.
- It's fine to think out loud. Half-formed answers are useful too.
- If the AI stays on the exercise when you raise something else, that's by design; save other topics for your therapist.
- Afterwards, keep the distortions list handy and jot down thoughts you notice during the week in the “My examples” column.

## What this session is not

So there are no surprises, here's what the AI **won't** do:

- It won't diagnose you or manage your treatment.
- It won't provide crisis support or emergency care.
- It won't carry out deeper TBCT techniques — those are introduced later by your therapist.
- It won't push solutions from other methods; it stays within this one exercise.

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**If you're in distress** This session is a learning exercise, not an emergency service. If at any point you feel overwhelmed, or you have thoughts of harming yourself, please pause and reach out to your study clinician or your local emergency services right away. Your safety matters more than finishing the exercise.

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*You don't need to prepare anything else — the AI will guide you step by step. When you're ready, begin whenever you like.*

## Cognitive Distortions List

Read the definitions and examples below so you can learn to recognise your own. During the week, write your own examples in the “My examples” column.

| Cognitive distortion                                                        | Definition                                                                                                                                      | Examples                                                                                                                                          | My examples |
|-----------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------|-------------|
| <b>1. Dichotomous thinking (all-or-nothing, black-and-white, polarized)</b> | I view a situation, a person or an event only in all-or-nothing terms, fitting them into only two extreme categories instead of on a continuum. | “I made a mistake, therefore I’m a failure.” “I ate more than I planned, so I blew my diet completely.”                                           |             |
| <b>2. Fortune telling (catastrophizing)</b>                                 | I predict the future in negative terms and believe that what will happen will be so awful that I will not be able to stand it.                  | “I will fail and this will be unbearable.” “I’ll be so upset that I won’t be able to concentrate for the exam.”                                   |             |
| <b>3. Discounting or disqualifying the positive</b>                         | I disqualify and discount positive experiences or events, insisting that they do not count.                                                     | “I passed the exam, but I was just lucky.” “Going to college is not a big deal, anyone can do it.”                                                |             |
| <b>4. Emotional reasoning</b>                                               | I believe my emotions reflect reality and let them guide my attitudes and judgments.                                                            | “I feel she loves me, so it must be true.” “I am terrified of airplanes, so flying must be dangerous.”                                            |             |
| <b>5. Labeling</b>                                                          | I put a fixed, global label, usually negative, on myself or others.                                                                             | “I’m a loser.” “He’s a rotten person.” “She’s a complete jerk.”                                                                                   |             |
| <b>6. Magnification / minimization</b>                                      | I evaluate myself, others, and situations magnifying the negatives and/or minimizing the positives.                                             | “I got a B. This proves how inferior I am.” “I got an A. It doesn’t mean I’m smart.”                                                              |             |
| <b>7. Selective abstraction (mental filter, tunnel vision)</b>              | I pay attention to one or a few details and fail to see the whole picture.                                                                      | “My boss liked my presentation, but he corrected a slide, so I know he didn’t mean it.” “One person found an error in my work, so I’ll be fired.” |             |
| <b>8. Mind reading</b>                                                      | I believe I know the thoughts or intentions of others (or that they know mine) without having sufficient evidence.                              | “He’s thinking that I failed.” “She thought I didn’t know the project.”                                                                           |             |
| <b>9. Overgeneralization</b>                                                | I take isolated cases and generalize them widely, using words such as “always,” “never,” “everyone.”                                            | “Every time I have a day off from work, it rains.” “You only pay attention to me when you want something.”                                        |             |

| Cognitive distortion                                         | Definition                                                                                                                                                     | Examples                                                                                                          | My examples |
|--------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------|-------------|
| <b>10. Personalizing</b>                                     | I assume that others' behaviors and external events concern (or are directed to) me, without considering other plausible explanations.                         | "I felt disrespected because the cashier didn't thank me" (though the cashier thanked no one).                    |             |
| <b>11. Should statements ("musts," "oughts," "have tos")</b> | I tell myself that events, people's behaviors, and my own attitudes "should" be the way I expected them to be and not as they really are.                      | "I should have been a better mother." "I shouldn't have made so many mistakes."                                   |             |
| <b>12. Jumping to conclusions</b>                            | I draw conclusions (negative or positive) from little or no confirmatory evidence.                                                                             | "As soon as I saw him I knew he had bad intentions." "He looked at me, so I concluded he blamed me."              |             |
| <b>13. Blaming (others or oneself)</b>                       | I direct my attention to others as sources of my negative feelings, failing to consider my own responsibility; or I take responsibility for others' behaviors. | "My parents are to blame for my unhappiness." "It's my fault that my son married a selfish person."               |             |
| <b>14. What if?</b>                                          | I keep asking myself questions such as "what if something happens?"                                                                                            | "What if my car crashes?" "What if I have a heart attack?" "What if my partner leaves me?"                        |             |
| <b>15. Unfair comparisons</b>                                | I compare myself with others who seem to do better than I do and place myself in a disadvantageous position.                                                   | "My father preferred my brother because he's smarter than I am." "I am a failure because she is more successful." |             |

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